

hovid-Folic Acid Tablet 5 mg

VIFOL05-var (MY)

DESCRIPTION

Round, yellow uncoated tablet, bevel-edged, flat-faces, "HD" and break-bar embossed on one face.

COMPOSITION

Each tablet contains: Folic Acid 5 mg.

PHARMACODYNAMICS

Folic acid is a member of the vitamin-B group. It is reduced in the body to tetrahydrofolate, which is a coenzyme for various metabolic processes including the synthesis of purine and pyrimidine nucleotides, including the synthesis of DNA; it is also involved in some amino-acid conversion and in the formation and utilization of formate.

Folic acid is necessary for normal erthropoiesis, including maturation of megaloblasts into normoblasts and nucleoprotein synthesis.

PHARMACOKINETICS

Absorption: Commercially available folic acid is almost completely absorbed from the gastrointestinal tract, even in the presence of malabsorption due to tropical sprue, although food folates are not.

Blood Concentration: Physiological blood concentrations are about 350 ng/ml

Half-life: Biological half-life, about 0.7 hours.

Distribution: Widely and rapidly distributed throughout the body and large amounts are stored in the liver, mainly as methylated folates; normal body stores are of the order 5 to 10mg, 50% of which is found in the liver.

Protein Binding: About 66% bound to plasma proteins.

Metabolic Reactions: Folic acid is converted to its metabolically active form (tetrahydrofolic acid) by hepatic dihydrofolate reductase in the liver.

Excretion: 20 to 90% of an ingested dose is excreted in the urine in 24 hours; the percentage of a dose excreted appears to be dose-dependent as after a 5mg - dose 50% is excreted in 24 hours and after a 15mg dose about 90% is excreted in 24 hours. It is excreted almost all as metabolites, however, if beyond excess daily needs, it is mainly in the unchanged form.

INDICATIONS

For the prevention and treatment of folate deficiency states.

For the prevention of neural tube defect in the foetus.

CONTRAINDICATIONS

Folic acid should never be given in the treatment of pernicious anaemia.

PRECAUTIONS

- Large and continuous doses of folic acid may lower the blood level of vitamin B12.
- Folic acid does not correct folate deficiency due to dihydrofolate reductase inhibitors; calcium folinate should be used instead.

MAIN SIDE/ADVERSE EFFECTS

No side effects have been reported with folic acid administration.

OVERDOSE

Folic acid is usually well tolerated.

DOSAGE AND ADMINISTRATION

Adults

Oral, initially 10-20 mg daily for 14 days or until haematopoietic response obtained.
Daily maintenance: 2.5 mg - 10 mg.

Child

Oral.
Up to 1 year: 250 mcg/kg daily
1 to 5 years: 2.5 mg/day
6-12 years: 5 mg/day

Pregnant

Oral, 5 mg daily starting before pregnancy and continued through the first trimester.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage:
Store below 30°C.
Protect from light and moisture.

Presentation/Packing:
Blisters of 10 x 10's.

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